

Patient voice inside the MA Code

A plain-language map of where engagement and compliance meet for industry partners. Done properly, genuine patient and carer engagement tends to meet the Medicines Australia Code as a byproduct. Built only to satisfy the Code, it fails on both counts.

For pharmaceutical and MedTech teams, the first question about patient engagement is often a compliance question. It is the right thing to ask and the wrong place to start. This briefing maps where patient and carer voice enters across the product lifecycle, what the Medicines Australia Code of Conduct expects of that engagement, and how to build it so the compliance takes care of itself. It is orientation, not legal advice.

The order that matters

Engagement designed primarily to satisfy the Code reads as exactly that, to consumers and to the bodies assessing your evidence. Engagement designed to genuinely inform a decision, and then documented carefully, meets the bar as a consequence. Build for authenticity first. Document as you go. The compliance follows.

Principle: compliance is the floor, not the design goal. If the engagement would still be worth doing with no Code at all, you are building it right.

Where patient voice enters across the lifecycle

Pre-launch: evidence and HTA

Patient and carer insight strengthens the value story and the HTA submission: what the condition actually costs people, which outcomes matter, where current care falls short. This is where credible, well-governed engagement does the most for both the science and the submission.

Launch

Insight shapes how a therapy is introduced to the people who will use it and the clinicians who prescribe it. The risk here is engagement that decorates a strategy already set, rather than informing it.

Post-launch

Real-world experience, access barriers, and outcomes that only show up in use. Ongoing, structured input keeps the patient reality in front of decisions rather than in a closed report.

What the Code expects of engagement

The Medicines Australia Code of Conduct governs how member companies interact with healthcare professionals and, relevantly here, with patients and consumer organisations. In plain terms, engagement should be:

EXPECTATION	WHAT IT MEANS IN PRACTICE
Legitimate purpose	A genuine need for the input, not a pretext for promotion or access.
Fair, documented value	Reasonable, recorded payment for genuine expertise and time, at fair market value.
Transparency	Arrangements and support disclosed; nothing that wouldn't survive being made public.
No disguised promotion	Engagement is not a channel to promote a product to consumers.
Independence respected	Patient organisations' independence and voice are not bought or steered.

This is a plain-language summary for orientation. The current Medicines Australia Code is the authority, and your medical and legal teams hold the final word on any specific arrangement.

Where it usually goes wrong

- **Starting with the Code, not the question.** The engagement is shaped to be defensible rather than useful, and it shows.
- **Commissioning too late.** Input is sought once the strategy is fixed, so it can only validate, never inform.
- **Confusing consultation with co-design.** Asking people to react to a finished plan, then calling it partnership.
- **Thin documentation.** Good engagement, poorly recorded, is hard to stand behind later.
- **Treating reimbursement as a grey area.** Pay fairly and record it. Underpaying skews who can take part as much as overpaying raises questions.

A pre-engagement checklist for industry teams

1. Can you name the decision this input will inform, and is it still open?
2. Is the purpose genuine, and would it survive being described publicly?
3. Have medical and compliance been involved from the design stage, not just sign-off?
4. Is payment fair, documented, and at market value for genuine expertise?
5. Are you protecting the independence of any patient organisation involved?
6. Is there a clear line between this engagement and any promotional activity?
7. Will participants hear what changed because of their input?
8. Have you defined the specific, measurable outcome the engagement is meant to move?

The bottom line: patient and carer reality should inform strategy, not decorate it. Build engagement that genuinely changes a decision, govern it properly, and the Code requirement is met along the way, not chased.

This briefing is general orientation, not legal or regulatory advice. Always refer to the current Medicines Australia Code of Conduct and your own medical, regulatory, and legal teams. · Kris Pierce Consulting · hello@krispierce.com.au · krispierce.com.au